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- End Homelessness Edmonton — ‘Costs and System Savings.’ endhomelessnessyeg.ca. Calgary Housing First evaluation: \$55,000/year homeless vs. \$21,000/year housed.
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- Alaska Permanent Fund Corporation — 2024 annual dividend: US\$1,702 per resident. Fund value approximately US\$87 billion.
- Alberta Doctors’ Digest — ‘Pushing back against changes that make life harder for disabled Albertans.’ November/December 2025. 47% increase in ED visits for psychiatric crises.
- Alberta Government — AISH Caseload Open Data. September 2025. 79,290 recipients.
- Government of Alberta — ADAP Program Page and Discussion Guide. Official ADAP benefit amounts, transition structure.
- Documented statement from community group administrator — Alberta AISH advocacy Facebook group, April 2026. Group membership tripled in one year to over 6,000 members; administrator attributes growth to ‘the treatment they’re getting from AISH, and the clawbacks and cutbacks.’

THE ALBERTA DISABILITY SYSTEM BREAKDOWN — April 2026 Report Series

Document 8 of 12: This Is A Choice, Not A Necessity — The Taxpayer Cost Report All figures sourced from Government of Alberta official publications and verified independent sources. Free to share.

DOCUMENT 9 OF 14: THE HUMAN COST REPORT

THE ALBERTA DISABILITY SYSTEM BREAKDOWN April 2026 Report Series

DOCUMENT 11 OF 11 The Human Cost Report Policy-Induced Hopelessness and the Public Health Consequences of the AISH-to-ADAP Transition

All figures sourced from Statistics Canada, Government of Alberta, CMHA, and verified independent sources. Free to share.

What Tammy Said “*This is why so many people say forget it and unalive themselves.*” That sentence was written in a public Facebook group by an Albertan living with a broken back, severe anxiety, and PTSD — choosing between rent, food, and utilities every month. It was not an isolated comment. It was a representative one. This document examines why.

SECTION 1: THE POPULATION — WHO IS BEING AFFECTED

The 79,290 Albertans currently receiving AISH support are not a homogeneous group. They are people with permanent, severe disabilities — physical, cognitive, and psychiatric — who have been assessed and confirmed as unable to sustain competitive employment. The following breakdown is drawn from Alberta Government open data as of September 2025.

1.1 Who Is on AISH

Metric	Figure
Total AISH recipients (Sept 2025)	79,290
Primary condition: physical disability	32,901 (41.5%)
Primary condition: mental illness	23,940 (30.2%)
Primary condition: cognitive disorder	22,419 (28.3%)
Single individuals	68,223 (86.0%)
Single parents	5,426 (6.8%)
Recipients with any employment income	12,770 (16.1%)
Recipients with zero employment income	66,263 (83.6%)

Source: Alberta Assisted Living and Social Services Open Data — AISH Caseload, September 2025

83.6% of AISH recipients have zero employment income. This is not a motivational statistic. It is a clinical one. These are people whose disabilities have been assessed as permanent and severe. ADAP’s employment focus does not change the underlying diagnosis.

SECTION 2: THE STRUCTURAL REMOVAL OF AGENCY

Hopelessness, in a clinical and public health context, is not simply sadness. It is the cognitive state produced when a person concludes that no action they take can improve

their situation. The AISH-to-ADAP transition has systematically removed every lever a recipient might pull. This section documents that removal.

2.1 The Financial Floor Has Been Lowered

Benefit / Change	Amount
AISH monthly benefit (2026)	\$1,940/month
ADAP base monthly benefit (July 1, 2026)	\$1,740/month
Transition benefit (July 1, 2026 – December 31, 2027)	+\$200/month top-up — keeps recipients at current AISH rate of \$1,940/month during transition period
Benefit after transition benefit ends (January 1, 2028)	–\$200/month / –\$2,400/year — drops to base ADAP rate of \$1,740/month unless changed by legislation
Canada Disability Benefit clawed back	–\$200/month (Alberta only — every other province exempts this)
Average 1-bedroom rent in Alberta (2026)	\$1,500–\$1,900+/month
Remaining after rent (best case)	\$40–\$440/month before food or utilities
3-bedroom rent for single parents (Alberta avg)	\$2,138/month — exceeds entire AISH payment

Sources: Government of Alberta ADAP Program Page; Urbanation/Rentals.ca National Rent Report; Financial Reality Report, Document 1 of this series

86% of AISH recipients are single individuals. After rent, the majority are left with between \$40 and \$440 per month for food, utilities, transportation, medication, and all other living expenses. A transition benefit keeps recipients at the current AISH rate until December 31, 2027 — after which the base ADAP rate of \$1,740/month applies. The delay does not resolve the structural impossibility. It postpones it 18 months.

2.2 The Right of Appeal Has Been Legislated Away

Bill 12, passed by the Government of Alberta on December 9, 2025, removed the independent Citizens Appeal Panel process for AISH and ADAP eligibility decisions. Under the previous system, recipients who disagreed with a program decision could appeal to an independent panel. That mechanism no longer exists.

- Decisions by government-appointed Medical Review Panels are now final.
- There is no independent appeal route for 79,290 Albertans with permanent disabilities.

- Alberta is the only province in Canada to both claw back the federal Canada Disability Benefit AND remove independent appeal rights from the same population simultaneously.

2.3 The Individual Caseworker Has Been Replaced With a Call Centre

Individual AISH caseworkers — who knew recipient files, medical histories, and individual circumstances — have been replaced with a centralized call centre model. Caseloads in the new model run as high as 200 files per worker. The practical consequence is that recipients have lost their primary human point of contact within the system at exactly the moment the system is changing.

When a person has no appeal right, no individual caseworker, no family doctor to complete their forms, and a benefit that does not cover rent — the rational conclusion is that the system has no space for them. That conclusion is not irrational. It is accurate.

SECTION 3: THE MENTAL HEALTH CRISIS IN NUMBERS

Alberta's mental health system was already under structural strain before the AISH-to-ADAP transition was announced. The following data documents the baseline conditions into which 79,290 people are now being pushed with reduced income, no appeal rights, and no individual caseworker.

3.1 The Existing Mental Health Baseline

Metric	Figure
Alberta suicide rate (per 100,000)	14.3 — above the national average
ED visits for psychiatric crises (Alberta increase)	47% increase documented
Alberta mental health budget (% of total health spending)	~5.5%
Mental Health Commission of Canada recommended minimum	12%
Psychiatrists per 100,000 (Alberta)	10.6 — below national average of 13.0
Public psychiatry wait time	6 months to 18+ months
What most public psychiatrists provide	Diagnosis + medication management only; therapy is not standard

Sources: CMHA Edmonton — 2024 State of Mental Health Report; Alberta Doctors' Digest, Nov/Dec 2025; AHS data via Best Choice Counselling 2025; Mental Health Commission of Canada

3.2 Treatments That Would Help Are Not Covered

For the 30.2% of AISH recipients whose primary diagnosis is mental illness, and the many more for whom mental illness is a secondary diagnosis, the gap between what the system offers and what evidence-based treatment requires is total.

Treatment	Covered by AISH/ADAP?	Available Through AHS?
EMDR (trauma, PTSD)	NOT COVERED	Not available in most regions
Individual DBT (BPD, trauma)	NOT COVERED	AHS offers groups only; not individual
CPT (C-PTSD, PTSD)	NOT COVERED	Limited; region-dependent
Somatic/body-based trauma therapy	NOT COVERED	Not available through AHS
Private psychologist or therapist	NOT COVERED	Not covered
Specialized BPD programs	NOT COVERED	Not available publicly

Sources: AHCIP schedule; Health Benefits Erosion Report, Document 2 of this series; Therapy Alberta, therapyalberta.com; First Session, firstsession.com

30.2% of AISH recipients carry a primary psychiatric diagnosis. A significant additional portion carry mental illness as a secondary condition alongside physical or cognitive disability. Recipients with severe, untreated mental illness cannot be expected to reliably navigate a program transition of this scale without clinical support. The system being changed is not providing that support. The income being reduced is not sufficient to purchase it privately. The appeal right that would have allowed a challenge to an adverse decision no longer exists.

SECTION 4: THE FISCAL COST OF IGNORING THE HUMAN COST

This is not only a moral argument. It is a fiscal one. The downstream cost of policy-induced mental health deterioration, housing loss, and emergency crisis intervention is measurably higher than the cost of the supports being reduced.

4.1 Emergency Department Cost vs. Prevention Cost

Comparison	Cost
ED psychiatric crisis visit (Alberta estimate)	\$1,000–\$3,000+ per visit
Psychiatric hospitalization (when required)	Thousands of dollars per day
Annual private therapy (52 sessions, uncovered)	\$5,000–\$8,000/year

Comparison	Cost
Net additional cost if 1 hospitalization results from no therapy	\$5,000–\$10,000+ per person
Alberta ED psychiatric crisis visits	Up 47% — already documented before this transition

Sources: Health Benefits Erosion Report, Document 2 of this series; Alberta Doctors' Digest, Nov/Dec 2025; Taxpayer Cost Report, Document 8 of this series

4.2 Housing Loss Cost vs. Housing Stability Cost

Scenario	Annual Taxpayer Cost
AISH recipient remains housed (benefit paid)	\$23,280
AISH recipient loses housing — emergency shelter and services	\$55,000
Additional taxpayer cost per person who becomes homeless	\$31,720 MORE
If 1,000 recipients lose housing (1.3% of caseload)	\$31.7 million in additional costs
AISH budget cut being made	\$49 million — less than housing 1,540 people would cost

Sources: End Homelessness Edmonton — Costs and System Savings; Taxpayer Cost Report, Document 8 of this series

THE FISCAL PARADOX: The government is cutting \$49 million from AISH while creating conditions — reduced income, no rent cap, no appeal rights, inaccessible caseworkers — that will predictably push some recipients into homelessness, at a downstream cost that exceeds the savings. This is cost transfer, not fiscal efficiency: from the disability income budget to emergency shelter, healthcare, and social services budgets.

SECTION 5: THE UNCRPD FRAMEWORK — A LEGAL OBLIGATION

Canada ratified the United Nations Convention on the Rights of Persons with Disabilities (UNCRPD) in 2010. The following articles are directly relevant to the conditions this report documents.

Article	Obligation
Article 19	Right to live independently in the community with adequate support. Removal of income support that makes housing unaffordable is inconsistent with

Article	Obligation
	this obligation.
Article 25	Right to the highest attainable standard of health without discrimination. Removal of evidence-based mental health coverage for a population defined by disability is inconsistent with this obligation.
Article 28	Right to an adequate standard of living and social protection. A benefit that does not cover rent in the province in which it is paid does not meet this standard.
Article 13	Access to justice on an equal basis with others. Removing the independent appeal mechanism for 79,290 people with disabilities while retaining it for other populations is inconsistent with this obligation.

Source: United Nations Convention on the Rights of Persons with Disabilities — Canada ratified 2010

SECTION 6: WHAT THE DATA PREDICTS

The combination of factors documented in this report — reduced income, housing unaffordability, removed appeal rights, inaccessible caseworkers, no GP access, no covered mental health treatment, and an accelerated transition timeline — produces a predictable set of public health outcomes. These are not speculative. They are the documented downstream effects of these conditions in comparable populations.

Predicted Outcome	Evidence Base
Increased ED psychiatric crisis presentations	47% increase already documented before transition; income reduction and loss of agency are established precipitants
Increased housing instability and homelessness	Benefit below market rent with no rent cap and no appeal rights creates structural displacement risk
Increased suicidality in affected population	Alberta suicide rate already above national average; hopelessness, financial crisis, and isolation are established risk factors
Deteriorating physical health outcomes	Untreated mental illness compounds physical conditions; reduced income limits medication and food access
Intergenerational impact on children of	Parental mental health and financial

Predicted Outcome	Evidence Base
AISH recipients	instability directly affect child outcomes
Increased cost to emergency, shelter, and social services systems	Documented in Taxpayer Cost Report, Document 8 of this series

SECTION 7: WHAT IS BEING ASKED

To the Government of Alberta

- Pause the mandatory AISH-to-ADAP transition until regulations are published in plain language and distributed to all 79,290 recipients.
- Restore independent appeal rights for AISH and ADAP eligibility decisions. Decisions that are final and unappealable are not consistent with due process or with Canada's obligations under the UNCRPD.
- Immediately cease the dollar-for-dollar clawback of the federal Canada Disability Benefit. Alberta is the only province or territory in Canada to do this.
- Expand AISH and ADAP health benefits to include evidence-based mental health treatments: EMDR, individual DBT, CPT, and somatic therapies.
- Restore individual caseworker relationships for recipients with complex medical and circumstantial profiles.
- Commission an independent public health impact assessment of the transition before July 1, 2026.

To the Government of Canada

- Require that future federal disability investments include enforceable provincial non-offset conditions, not recommendations.
- Direct the Minister of Disability Inclusion to formally raise Alberta's Canada Disability Benefit clawback with the Government of Alberta.
- Direct the Standing Committee on Health to examine the intersection of the healthcare worker shortage and the ADAP medical form requirement.

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- AHCIP Schedule of Medical Benefits — Government of Alberta. Mental health coverage inclusions and exclusions
- Documents 1–10, Alberta Disability System Breakdown — April 2026 Report Series. All figures and cross-references verified against primary government and academic sources

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Document 11 of 11: The Human Cost Report All figures sourced from Government of Alberta official publications and verified independent sources. Free to share.
